

The right medicine for the world economy

The pandemic threatens an economic crisis as well as a health crisis. Both need fixing

IT IS NOT a fair fight, but it is a fight that many countries will face all the same. Left to itself, the covid-19 pandemic doubles every five to six days. When you get your next issue of *The Economist* the outbreak could in theory have infected twice as many people as today. Governments can slow that ferocious pace, but bureaucratic time is not the same as virus time. And at the moment governments across the world are being left flat-footed.

The disease is in 85 countries and territories, up from 50 a week earlier. Over 95,000 cases and 3,200 deaths have been recorded. Yet our analysis, based on patterns of travel to and from China, suggests that many countries which have spotted tens of cases have hundreds more circulating undetected (see Graphic detail). Iran, South Korea and Italy are exporting the virus. America has registered 159 cases in 14 states but as of March 1st it had, indefensibly, tested just 472 people when South Korea was testing 10,000 a day. Now that America is looking, it is sure to find scores of infections—and possibly unearth a runaway epidemic.

Wherever the virus takes hold, containing it and mitigating its effects will involve more than doctors and paramedics. The World Health Organisation has distilled lessons from China for how health-care systems should cope (see Briefing). The same thinking is needed across the government, especially over how to protect people and companies as supply chains fracture and the worried and the ill shut themselves away.

The first task is to get manpower and money to hospitals. China drafted in 40,000 health workers to Hubei province. Britain may bring medics out of retirement. This week the World Bank made \$12bn and the IMF \$50bn available for covid-19. The Global Fund, which fights diseases like malaria and TB, said countries can switch grants. In America Congress is allocating \$8.3bn of funding. The country has some of the world's most advanced hospitals, but its fragmented health system has little spare capacity. Much more money will be needed.

Just as important is to slow the spread of the disease by getting patients to come forward for testing when outbreaks are small and possible to contain. They may be deterred in many countries, including much of America, where 28m people are without health coverage and many more have to pay for a large slug of their own treatment. People also need to isolate themselves if they have mild symptoms, as about 80% of them will. Here sick pay matters, because many people cannot afford to miss work. In America a quarter of employees have no access to paid sick leave and only scattered states and cities offer sickness benefits. Often the self-employed, a fifth of Italy's workforce, do not qualify. One study found that, in epidemics, guaranteed sick pay cuts the spread of flu in America by 40%.

Sick pay also helps soften the blow to demand which, along with a supply shock and a general panic, is hitting economies. These three factors, as China shows, can have a dramatic effect on output. Manufacturing activity there sank in February to its lowest level since managers were first surveyed in 2004. In the quarter to March the economy as a whole could shrink for the first time since the death of Mao Zedong. The OECD expects glo-

bal growth this year to be its slowest since 2009. Modelling by academics at the Australian National University suggests that GDP in America and Europe would be 2% lower than it would have been in the absence of a pandemic and perhaps as much as 8% lower if the rate of deaths is many times higher than expected. Financial markets are pricing in fear. The S&P 500 has fallen by 8% from its peak on February 19th. Issuance of corporate debt on Wall Street has more or less stopped. The yield on ten-year Treasuries dipped below 1% for the first time ever.

In rich countries, most of the economic effort has been directed towards calming financial markets. On March 3rd America's Federal Reserve cut rates a fortnight before its monetary-policy meeting, and by an unusually large half-a-percentage point (see Finance section). The central banks of Australia, Canada and Indonesia have also acted. The Bank of England and the European Central Bank are both expected to loosen policy, too.

Yet this slowdown is not a textbook downturn. Lower rates will ease borrowing costs and shore up sentiment, but no amount of cheap credit can stop people falling ill. Monetary policy cannot repair broken supply chains or tempt anxious people into venturing out. These obvious limitations help explain why stockmarkets failed to revive after the Fed's cut.

Better to support the economy directly, by helping affected people and firms pay bills and borrow money if they need it. For individuals, the priority should be paying for health care and providing paid sick leave. The Trump administration is considering paying some hospital bills for those with the virus. Japan's government will cover the wages of parents who stay at home to care for children or sick relatives; Singapore's will help cab drivers and bosses whose employees are struck down.

More such ideas will be needed.

For companies the big challenge will be liquidity. And although this shock is unlike the financial crisis, when the poison spread from within, that period did show how to cope with a liquidity crunch. Firms that lose revenues will still face tax, wage and interest bills. Easing that burden, for as long as the epidemic lasts, can avoid needless bankruptcies and lay-offs. Temporary relief on tax and wage costs can help. Employers can be encouraged to choose shorter hours for all their staff over lay-offs for some of them. Authorities could fund banks to lend to firms that are suffering, as they did during the financial crisis and as China is doing today. China is also ordering banks to go easy on delinquent borrowers. Western governments cannot do that, but it is in the interest of lenders everywhere to show forbearance towards borrowers facing a cash squeeze, much as banks did to public-sector employees during America's government shutdown in 2018-19.

There is a tension. Health policy aims to spare hospitals by lowering the epidemic's peak so that it is less intense, if longer-lasting. Economic policy, by contrast, aims to minimise how long factories are shut and staff absent. Eventually governments will have to strike a balance. Today, however, they are so far behind the epidemic that the priority must be to slow its spread. ■



▶ also to reinvigorate their faltering alliance.

Some NATO members are reluctant to work with Mr Erdogan. This is understandable. He locks up dissidents, cosies up to Russia, uses refugees as bargaining chips, and is often at odds with NATO. Look no further than Syria, where Turkey has pounded Kurdish fighters, whom it calls terrorists, but whom America regards as allies in the fight against Islamic State. Mr Erdogan sees Vladimir Putin, Russia's president, as the man to deal with in Syria—and elsewhere. Mr Putin, in turn, uses Mr Erdogan to divide and weaken NATO. Russia's sale to Turkey of an S-400 anti-aircraft system, which is incompatible with NATO systems, has deepened the rift between Turkey and its allies.

But Turkey is too important for NATO to abandon. It has the alliance's second-biggest army and sits at the crossroads between west and east. The situation in Idlib, dire as it is, provides an opportunity to reset relations. Turkey has renewed its call for NATO to patrol a no-fly zone over the province and asked America for Patriot missiles to defend itself against attack. Western leaders

are understandably reluctant to put troops on the ground or planes in the air around Idlib. NATO, though, can do more than merely express solidarity. Promises of more ammunition, humanitarian aid and radar surveillance over Syria are a start. Spain already stations Patriot missiles in Turkey and other European allies should step in. NATO should put its weight behind talks to reach a political settlement over Idlib. And Turkey should insist that some of its allies should be present at such talks, something Russia rejects.

These moves will not completely heal the rift between Turkey and the West, nor will they solve the problem of Idlib, which has been festering for years. But they might restore some of Turkey's faith in its allies and send a message to Mr Putin. The fact that such strategic considerations—and a fear of Syrian refugees—might motivate Western policymakers more than their concern over struggling Syrians is disheartening. Yet NATO attention would look the same to the people of Idlib whatever its motive. Helping Turkey there would alleviate some of their suffering. ■

Britain's budget

Be bold, Boris

Covid-19 is not the only reason that the government should increase spending

THE SMOKE signals rising from Downing Street suggest that the budget due on March 11th will be dull. The covid-19 virus has clouded the outlook; the new chancellor, Rishi Sunak, needs to get his feet under the desk; there will be more fiscal announcements later in the year. It sounds as though the government plans to treat the budget as a holding operation. That would be a mistake. A new government's first budget is a big moment, and this government has a lot to do. It is time to be bold, not boring.

For over a decade economic policy has been about managing a series of blows. First there was the financial crisis, which landed the government with the cost of rescuing the banks. Then there was Brexit, which has delayed firms' investment decisions. Now there is covid-19, which is bound to slow the economy further.

These last two factors would argue for an expansionary budget. But there is a further reason: while the government managed immediate perils, long-term problems—notably poor productivity and public services, particularly outside the south-east—have gone unattended.

To deal with these issues, Mr Sunak will have to spend more in two areas. The first is investment. The government looks set to double the R&D budget and boost infrastructure spending, particularly for greener transport. That is all to the good, but other sorts of investment also need a boost. To enhance workers' skills, Britain should spend more on technical education for school leavers and those already in work. And to raise private investment, which has been weak since 2016 and is growing more slowly than in any other G7 economy, the chancellor should increase capital allowances, which let investment spending be deducted from tax bills, and thus encourage firms to invest.

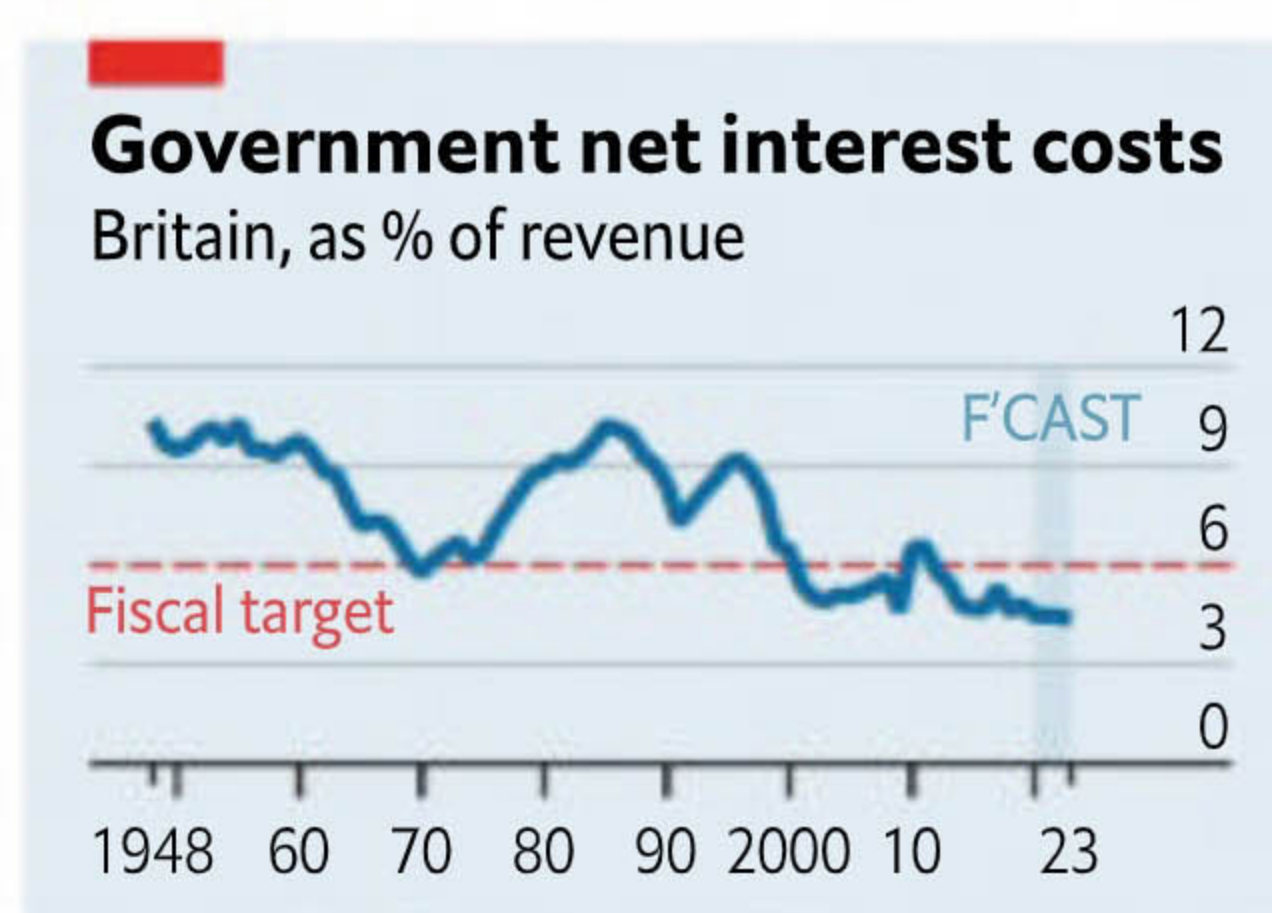
The second area that deserves more cash is public services. Local authorities, which provide many of them, have about a third less to spend, in real terms, than they did in 2010, and the government's current plans imply further cuts. That is poor poli-

tics and poor economics. A shiny new railway station, paid for out of the capital budget, will be scant consolation to many voters, nor will it boost poorer regions' prospects if local services, funded out of current spending, continue to deteriorate.

How to pay for all this? In the long run, modestly higher public spending will require higher taxes. And some taxes should rise now, not least to show voters the direction of reform. Raising fuel duty, which has been frozen for almost a decade, will bring in revenue and help wean Britain off fossil fuels. Council tax, the main charge on residential property, needs an overhaul both to raise money and to remove its regional distortions. The property valuations on which it is set date back to 1991, since when real house prices have quadrupled in the capital and doubled in Yorkshire. People sitting on multi-million-pound London piles pay less property tax than those in cheaper homes in other bits of Britain.

Most of the extra spending, however, should be funded by borrowing. That alarms many sensible Britons, who worry that the government is getting too far into debt and will lose credibility by, yet again, breaking the fiscal rules it has set itself. Those fiscal rules have been revised five times over the past decade.

These are valid concerns, but they are outweighed by Britain's situation. The amount of debt matters less than the cost of servicing it, and, with interest rates historically low, that has fallen steadily from 6.5% of tax revenues in 2010 to 4.6% now. Britain is also insulated against shocks because the government's bonds, at a 15-year average maturity, are longer-dated than those of any other major economy. Of Britain's three fiscal rules—to balance the current budget in three years, to keep capital spending below 3% of GDP and to keep an eye on government borrowing costs—the one that really matters, therefore, is a pledge to keep the government's interest bill below 6% of tax revenues. Time to junk the other rules, stick to that one, and start spending. ■





New world curriculum

BEIJING, MILAN, SEOUL, SINGAPORE AND WASHINGTON

With just four countries still accounting for 97% of covid-19 cases, the rest of the world needs to learn some lessons

SPEAKING FROM his home in Abuja, Chikwe Ihekweazu, the director general of the Nigerian Centre for Disease Control, says China has one big lesson to teach the world about covid-19: "It can be contained." Mr Ihekweazu was in voluntary self-isolation following a visit to Wuhan, the city at the centre of the Chinese outbreak, as part of a group of experts dispatched by the World Health Organisation (WHO) to examine the country's measures to stop the spread of covid-19.

These measures seem to have had significant success. On February 4th China recorded 3,887 new cases. On March 4th the number was 139. The report that the WHO group published on February 28th put the good results down to the way that the state had used manpower and technology to implement quarantine meticulously on an unprecedented scale. It did not go into the disturbing mechanisms of state power employed to that end.

All over the country cities closed down schools, public transport and almost all social and economic activity to stop people

from moving around. In Wuhan, a city of 11m, people have been restricted to their homes for five weeks. The lockdown was enforced not just by the network of officials which covers every block of flats, street and alley but also, under the influence of those officials, by the property managers at residential compounds. In some villages life is now relatively unconstrained; there are reports that in others every visitor has to be cleared by a local party boss. The top-down set-up lends itself to excessive alacrity among the lower echelons. Missing a target is less dangerous than overstepping the mark. Hence the (swiftly censored) online videos showing police smashing Mahjong tables, people being paraded or tied to trees for failing to wear face-masks, or a screaming woman barricaded in her home with iron bars.

How less thuggish states can achieve similar results remains to be seen. In South Korea, the country with the most cases outside China, stringent measures widely supported by the public may be slowing the rate of spread. A drop has definitely been

seen in Singapore, where pandemic preparedness became a priority after the SARS outbreak of 2003. The city-state knows where all but 12 of the 112 cases it has seen got infected.

But whether the isolation of whole towns in northern Italy has done much to curb the outbreak there remains to be seen. In Iran there is no evidence as yet of any overarching control at all. And in America the response to date has been a shambolic missed opportunity. Shockingly, the worst American bungling has more in common with the catastrophic early stages of the Chinese epidemic—when officials minimised risks and punished truth-tellers, thus letting the disease spread much further and faster than it might have—than with the country's later co-ordinated control efforts.

The wrong controls

In the week from February 26th to March 4th the number of countries and territories reporting cases of covid-19 rose from 50 to 85 (see map on next page). Most, so far, have fewer than a dozen cases, mostly imported from or directly linked to cases in China, Iran, Italy and South Korea, which between them accounted for 91,903 of the 95,124 confirmed cases as of March 4th (see chart on next page). Low case numbers, though, should not be taken to signify successful containment; they are often a measure of ignorance.

Take America. On February 25th Larry ►►

► Kudlow, chief economic adviser to President Donald Trump, told reporters that “We have contained this. I won’t say airtight, but it’s pretty close to airtight.” As he spoke a cluster of cases at a care facility in Washington state was showing that America’s public-health agencies had been caught flat footed. Test kits made available by the Centres for Disease Control and Prevention (CDC) were faulty; restrictions were limiting tests in other settings. By March 1st, when South Korea had run 100,000 tests for the virus, America—which saw its first case on January 23rd—had run fewer than 500.

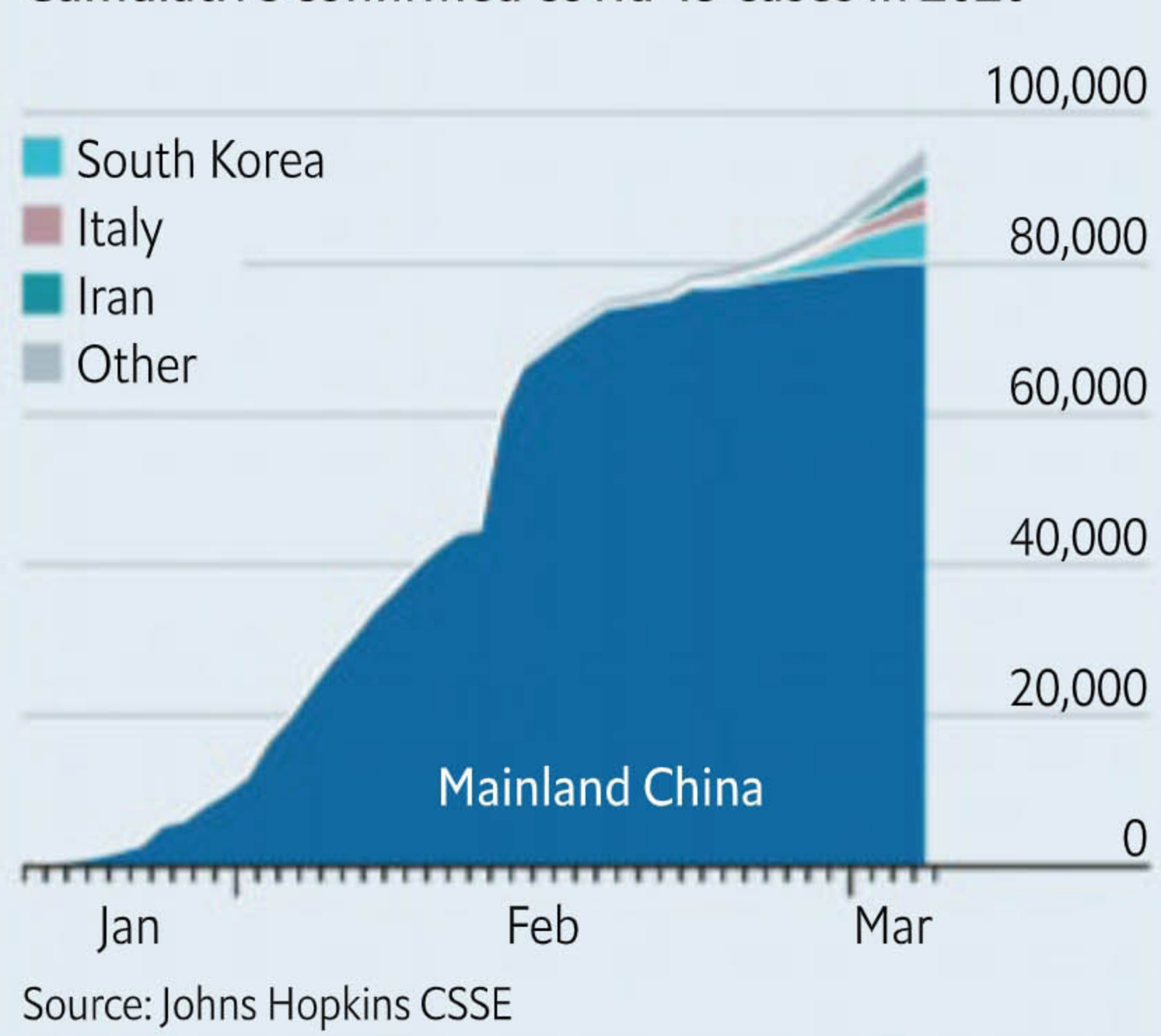
The WHO team urged all countries which have seen any cases to follow China in carrying out as much testing as they could, especially of unexplained pneumonia cases and flu-like symptoms in patients who seem not to be infected with the flu virus. “Test and you shall find,” says Gabriel Leung of the University of Hong Kong, who was also on the WHO team. “You either test and find it early, and do something about it, or the body bags are going to pile up,” he adds.

That dire warning has not yet born bitter fruit in under-tested America, but it could. As of March 4th, there were cases in 14 American states. Some of them, such as that of a New York lawyer and his family, have no obvious connection to any of the four high-incidence countries, strongly suggesting that there are what epidemiologists call silent chains of transmission in the country. In a population without measures in place to control such chains, a single undiagnosed case can, in principle, give rise to more than 3,000 cases six weeks later. On March 4th, while Mr Trump blamed his predecessor, Barack Obama, for the CDC’s problems, Andrew Cuomo, the governor of New York, revealed he had asked 1,000 people in Westchester, where the infected lawyer lives, to isolate themselves.

In South Korea, by contrast, the government is being forthright and formidably

The iceberg gets its tip

Cumulative confirmed covid-19 cases in 2020



transparent, allowing Koreans to trace their possible brushes with the disease. As well as briefing the press thoroughly twice a day, and texting reporters details of every death, the government puts online a detailed record of each new patient’s movements over previous days and weeks, allowing people to choose to shun the places they visited. The risk of illicit activity being thus uncovered—at least one extramarital affair may have been—gives people an extra incentive to avoid exposure to a disease which, in most of the infected, results in only mild symptoms.

No Roman candles

Across the country, schools are closed and public gatherings cancelled. Though neither Daegu nor Cheongdo, the cities which saw the first clustered outbreaks, were ever completely locked down, more than 9,000 people in Daegu were quarantined. As in China, where personal data from electronic payment and social media apps have been used by the authorities to track people’s movements and estimate their chances of infection, phones play a role in South Korea, too. The government has produced an app which reports on quaran-

ted people’s movements and can alert authorities if they abscond. Officials call regularly to check in on people.

South Korea has powers that put very little weight on its citizens’ privacy; some aspects of its response might be hard to mount in other democracies. But in countries where there is a high level of trust in government, some strong measures will be supported. When SARS hit Toronto in 2003 only 23 of almost 30,000 people placed under quarantine refused to comply. Asked in a survey how serious a crime or misdemeanour they thought breaking quarantine was, half of Canadians ranked it as similar to manslaughter. In all the countries questioned about their attitudes to quarantine in an Ipsos MORI poll at the end of February, a majority said that they would back quarantines if the government called for them. In Canada, the majority was 78%, one of the largest seen. In Italy it was only 60%, the smallest.

That seems to reflect national levels of trust. Last June polling by the Wellcome Trust, a charity, found that 78% of Canadians said they trusted government health advice. In Germany and Britain the number was over 80% and in South Korea a remarkable 86%. In Italy it was just 63%; in America, worryingly, lower still at 59%. Italians will need to wait a bit to discover if their government’s response has deserved what trust they have accorded it. According to Walter Ricciardi, a leading public-health expert in Rome, it will take until the middle of March to know whether sealing towns with outbreaks off into two “red zones” has worked to slow the virus’s spread. It cannot be good news that over 400 cases have now been diagnosed in an area around the city of Bergamo, north of Milan. On March 4th the government announced it was closing schools and universities until March 15th.

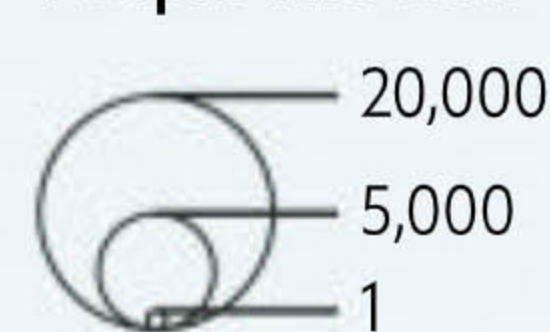
If a quarantine is to enjoy public support, the people it shuts away need to be well looked after. Economically, those in Italy may be: the government has put aside €3.6bn (\$4bn) to cover the epidemic’s costs. Some of it is earmarked to help exporters such as the doubly unlucky Erik Granzon. Not only is his fireworks company suffering the inconvenience of life in one of the two red zones; it also lost €300,000 last week when big displays were cancelled in Qatar and Milan so as to limit opportunities for further infection.

Some in the other red zone have more pressing problems. In a video posted to YouTube on March 3rd, Costantino Pesatori, the mayor of Castiglione d’Adda, a village of 4,650 inhabitants, said: “We feel truly alone. People are ill and are not receiving the necessary assistance. Help us please.” There are around 100 infections among the villagers; there have been 15 deaths. Mr Pesatori told *The Economist* that all three doctors in the village were in quar-

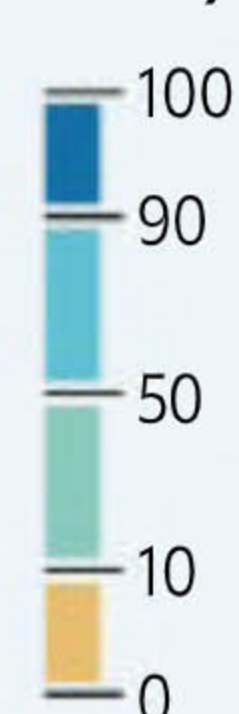
Pockmarked planet

Confirmed covid-19 cases, to March 4th 2020

People infected



Share of cases that occurred in the past week, %



Source: Johns Hopkins CSSE



After a seriously thorough search,
you've found the perfect bike helmet
for your little one, delivered to
your door in only one day.

We put a lot of thought into
safety too. That's why we've always
had a whole host of safety initiatives
in our Fulfilment Centres. Last year
we spent 238,000 hours on health
and safety training across the UK.
Because you know what they say,
you can never be too careful.

It's just one of the things we do
to create a great place to work.
Find out more about Amazon
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▶ antine and a locum had “no gloves, no mask, no overalls—nothing to protect him.” He is begging the authorities to open consulting rooms at two local hospitals or send in army doctors.

A lack of supplies is being reported elsewhere in the red zones, too. In nearby Casalpuusterlengo, Anna Rossi, a pharmacist, says she has no disinfectant gel to sell her customers, nor any masks capable of stopping contagion. On March 2nd a group of general practitioners wrote to the health minister in Rome that they too lacked protective equipment. “We want to stay at our patients’ sides,” they wrote. “But how can we do so effectively if we are not given the minimal means with which to defend them and defend ourselves?”

This looks like being a problem more widely. The WHO says the world’s health-care workers will need 89m medical masks a month to deal with covid-19. Because production is currently only about 70% of that there are shortages, and prices have gone up six-fold. Germany, Russia, Taiwan and Thailand have restricted exports of masks to protect their domestic supplies. China, which last year made half the world’s face masks, has not banned exports. But in practice they have, understandably, ground to a halt.

Masks of sorrows

Production, though, has rocketed—as it could elsewhere, if politicians forced it to. According to the government, China is now making 116m face masks a day, 12 times the production a month ago, with 1.7m of them the high-performance sort that health-care workers need when faced with patients coughing and sneezing. A General Motors joint venture in south-western China is making both its own disposable face masks and face-mask-making machinery for the many other companies doing the same. There are no precise figures for the production of tests, but the number carried out suggests that it, too, has soared.

Some populations will not be able to afford to test at the necessary rates—or to hospitalise large numbers of patients. In Africa, which currently has very few cases, a big emphasis on tracking cases at this early stage in the epidemic could serve to limit the absolute number of infections, avoiding wherever possible the extensive community transmission that seems to have taken off in Iran. Bitter experience of other epidemics means that some African countries are well set up for this. Mr Ihekweazu’s teams go out to investigate a new disease outbreak of some sort almost every week with the help of SORMAS, an open-source data-logging app used by health workers.

That said, although covid-19 has become a major priority, it is not the overriding concern it could be. Mr Ihekweazu notes that Nigeria is currently experienc-

ing the largest spike in Lassa fever cases it has ever seen; Africa also has a high burden of the world’s most deadly respiratory disease, tuberculosis. Most of Mr Ihekweazu’s team is focused on the new virus, but some remain ring-fenced for other areas “so that we don’t drop the ball.”

In places where the covid-19 ball is being dropped, or has been dropped already, the sort of self-isolation being urged in Westchester will be crucial to slowing the spread of the disease. In Britain statutory sick pay, normally not applicable until the fourth day of absence, will now be paid from the first to encourage people to stay at home if they think they need to. That said, the pay is not generous and does not cover the lowest paid.

In America both the health-care system and paid-leave laws are poorly configured for epidemics. Public-health authority is mostly devolved to state and city health agencies, creating a patchwork system of control. Preparedness, funding and sophistication all vary by county and hospitals, meaning that response and care could become uneven.

The patchwork system could be especially problematic if it leads to sick people showing up to work when they should not. Few lower-income workers in service-sector jobs have the option of paid sick leave available to them. The prospect of lost wages—and potentially a lost job—could be enough to persuade them to attend work rather than stay at home feeling a bit ill but increasingly poor. A study of paid sick-leave mandates in America by Stefan Pichler and Nicholas Ziebarth, two economists, found that the policy reduced the spread of influenza by 5% in normal times and 40% during a wave. Assuming that covid-19 is comparable, that calculation suggests that the 39 states that do not require paid sick leave risk a worsened crisis.

The second big problem is that, because

pandemic preparedness is not generally billable, American hospitals do not keep spare infectious-disease capacity. “We’re already running our health-care systems razor-thin in terms of capacity. There’s very little excess capacity in people, beds...or protective equipment,” says Michael Osterholm, an infectious-disease specialist at the University of Minnesota. A lack of preparedness has already forced the quarantine of 124 staff because of a single patient in California.

At the patient level, the costs of medical testing and treatment could act as a hindrance to effective containment. There are already widely circulating reports of worried patients showing up to emergency rooms for testing who are leaving with medical bills for thousands of dollars. More than a quarter of the 28m uninsured Americans say they have put off medical care they cannot afford. The Trump administration is said to be considering routing disaster-relief funds towards reimbursing the uninsured, which would ease things. The implications for the un- and under-insured of the \$8.3bn “supplemental appropriation” passed by the House on March 4th are not clear.

Pay attention

Not all countries can be as well prepared as Singapore: rich enough to pledge S\$5.6bn (\$4bn)—as much as Italy—to blunt the disease’s economic impacts, well organised enough to enforce pretty strict measures and shaken by SARS into taking such things seriously. Yet Lee Hsien Loong, the prime minister, still worried in February that the costs would eclipse those of SARS. Lawrence Wong, co-chair of the ministerial task force on covid-19, warns that “all it takes is one incident, one event, and there is a sharp spike in cases and sustained transmission.” Everyone has things to learn. Some have more than others. ■

